

Chapter 1: Introduction

Welcome

Welcome to the UHC AZ Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 plan manual. This comprehensive and up-to-date reference PDF manual allows you and your staff to find important information such as processing a claim and prior authorization. This care provider manual explains the policies and procedures of the UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs network. This manual also includes important phone numbers and websites. We hope it provides you and your office staff with helpful information and guide you in making the best decisions for your patients.

Click to access different care provider manuals

- **Administrative guide – UHCprovider.com/guides**
 - Under UnitedHealthcare Care Provider Administrative Guide for Commercial, Medicare Advantage (including Dual Complete Special Needs Plans), click on View Guide. Some states may also have Medicare Advantage information in their Community Plan manual.
- **A different Community Plan care provider manual**
 - UHCprovider.com/guides Under Community Plan Care Provider Manuals for Medicaid Plans by State, click on **Your State**

If you have questions about the information or material in this manual or about any of our policies, please call **Provider Services (ACC/DD): 1-800-445-1638**. We greatly appreciate your participation in our program and the care you offer our members.

Important information about the use of this care provider manual

In the event of a conflict between your agreement and this care provider manual, the manual controls unless the agreement dictates otherwise.

In the event of a conflict between your agreement, this manual and applicable federal and state statutes and regulations and/or state contracts, applicable federal and state statutes and regulations and/or state contracts will control. UnitedHealthcare Community Plan reserves the right to supplement this manual to help ensure its terms and conditions remain in compliance with relevant federal and state statutes and regulations.

This care provider manual will be amended as policies change.

Background

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 are Medicare Advantage Special Needs Plans, serving members who are dually eligible for Medicare and Arizona Health Care Cost Containment System (AHCCCS) (Medicaid) within the UHC Dual Complete AZ-S001 programs service area.

Members of UHC Dual Complete AZ-S001 programs have already demonstrated eligibility for and been enrolled in Medicare Part A, Medicare Part B, and AHCCCS Medicaid Title XIX benefits. UHC Dual Complete AZ-S001 programs members may be enrolled in UnitedHealthcare Community Plan.

For Division of Developmental Disabilities members who are dual eligible, UHC Dual Complete AZ-S001 is available statewide. For Arizona Long Term Care Elderly Physically Disabled (EPD) members who are dual eligible, UHC Dual Complete AZ-Y001 is available in: Maricopa, Pinal, Gila, Apache, Coconino, Navajo, Mohave, and Yavapai counties. UnitedHealthcare Community Plan provides AHCCCS programs for AHCCCS Complete Care (ACC), Arizona Long Term Care Elderly Physically Disabled (ALTCS EPD) and Developmental Disabilities (DD).

Contacting UHC Community Plan Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs manage a comprehensive care provider network of independent practitioners and facilities across Arizona. The network includes health care professionals such as primary care providers (PCPs), specialist care providers, medical facilities, allied health professionals, and ancillary service providers. UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs offer several options to support care providers who require assistance.

Electronic data interchange online resources

UHCprovider.com is your home for care provider information with access to Electronic Data Interchange (EDI), Provider Portal online services, medical policies, news bulletins. It also includes great resources to support administrative tasks such as eligibility, claims, claims status and prior authorizations and notifications. Go to [Self Service](#) for Self Service Tool online training and information. See [EDI Transaction Support Form](#).

Electronic data interchange

Electronic data interchanges (EDI) is an online resource using your internal practice management or hospital information system to exchange transactions with us through a clearinghouse.

The benefit of using EDI is it permits care providers to send batch transactions for multiple members and multiple payers in lieu of logging into different payer websites to manually request information. This is why EDI is usually care providers' and UnitedHealthcare Community Plan's first choice for electronic transactions.

- Send and receive information faster
- Identify submission errors immediately and avoid processing delays
- Exchange information with multiple payers

- Reduce paper, postal costs and mail time
- Cut administrative expenses
- EDI transactions available to care providers are:
 - 270/271: Eligibility and benefit inquiry and response
 - 275: Claim attachments
 - 276/277: Claim status inquiry and response
 - 278: Authorization and referral request
 - 278I: Prior authorization and notification inquiry
 - 278N: Hospital admission notification
 - 835: Electronic remittance advice (ERA)
 - 837D: Dental claim
 - 837I: Institutional claim
 - 837P: Professional claim or vision claim

Visit UHCprovider.com/edi >

[EDI transaction and code sets](#) for more information.

Getting started

- If you have a practice management or hospital information system, contact your software vendor for instructions on how to use EDI in your system
- Contact clearinghouses to review which electronic transactions can interact with your software system

Read our [Clearinghouse Options](#) page for more information.

UnitedHealthcare Provider Portal

Access patient- and practice-specific information 24/7 within the [UnitedHealthcare Provider Portal](#). You can complete tasks online, get updates on claims, reconsiderations, appeals and payment detail, submit prior authorization requests, check eligibility and update your practice demographic information – all at no cost without calling.

See [UnitedHealthcare Provider Portal](#) for access and to create ID or sign in using a One Healthcare ID.

- If you already have a One Healthcare ID, simply go to the [UnitedHealthcare Provider Portal](#) to access
- If you need to set up an account on the portal, follow [these steps](#) to register

- Access remittance advice and review recoveries
- Review your preventive health measure report
- Access the Early and Periodic Screening, Diagnosis, and Treatment (EPSDT) toolset
- Search for CPT codes. Type the CPT code in the header search box titled “What can I help you find?” on UHCprovider.com, and the search results will display all documents and/or web pages containing that code
- Find certain web pages more quickly using direct URLs. You’ll see changes in the way we direct you to specific web pages on our **UnitedHealthcare Provider Portal**. You can now use certain direct URLs, which helps you find and remember specific web pages easily and quickly. You can access our most used and popular web pages on UHCprovider.com by typing in that page’s direct URL identified by a forward slash in the web address, e.g. UHCprovider.com/claims. When you see that forward slash in our web links, you can copy the direct URL into your web page address bar to quickly access that page.

You will conduct business with us electronically. Using electronic transactions is fast, efficient, and supports a paperless work environment. Use both EDI and **UnitedHealthcare Provider Portal** for maximum efficiency in conducting business electronically.

Here are the most frequently used transactions on the **UnitedHealthcare Provider Portal**:

- **Eligibility and Benefits** – View patient eligibility and benefits information for most benefit plans. For more information, go to UHCprovider.com/eligibility.
- **Claims** – Get claims information for many UnitedHealthcare plans, including access letters, remittance advice documents and reimbursement policies. For more information, go to UHCprovider.com/claims.
- **Prior Authorization and Notification** – Submit notification and prior authorization requests. For more information, go to UHCprovider.com/priorauth.
- **Specialty Pharmacy Transactions** – Submit notification and prior authorization requests for certain medical injectable specialty drugs. see UHCprovider.com/pharmacy for more information.

- **My Practice Profile** – View and update your provider demographic data that UnitedHealthcare members see for your practice. For more information, go to UHCprovider.com/mypracticeprofile.
- **Document Library** – Access claim letters for viewing, printing, or download. The Document Library Roster provides member contact information in a PDF and can only be pulled at the individual practitioner level. For more information, go to UHCprovider.com/documentlibrary.

See **UnitedHealthcare Provider Portal** to learn more about the available self-paced user guides for various tools/tasks.

Direct connect

Direct connect is a free online portal that lets you securely communicate with payers to address errant claims. This portal can replace previous methods of letters, faxes, phone calls and spreadsheets. It also helps:

- Manage overpayments in a controlled process
- Create a transparent view between care provider and payer
- Avoid duplicate recoupment and returned checks
- Decrease resolution timeframes
- Real-time reporting to track statuses of inventories in resolution process
- Provide control over financial resolution methods

All users will access Direct Connect using the **UnitedHealthcare Provider Portal**. You will also find the [optum-direct-connect-fact-sheet.pdf](#) on this portal.

Email directconnectsupport@optum.com to get started with Direct Connect.

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 Roster

PCPs are given access to a roster of all assigned members. PCPs should use this to determine if they are responsible for providing primary care to a particular member. Rosters can be viewed electronically on UHCprovider.com.

Additional resources

Provider Services

Provider Services is the primary contact for care providers who need help. It is staffed with representatives trained specifically for UnitedHealthcare Community Plan.

They can answer your questions about Medicaid benefits, eligibility, claim decision, forms required to report specific services, billing questions and more. Provider Services works closely with all departments in UnitedHealthcare Community Plan.

- UnitedHealthcare Community Plan **Provider Services** (ACC/DD): **1-800-445-1638**
- Long-Term Care (ALTCES EPD): **1-800-293-3740**

Provider Services works closely with all departments in UnitedHealthcare Community Plan.

Office of Individual and Family Affairs

The AHCCCS Office of Individual and Family Affairs (OIFA) works closely with you, our members and their loved ones to help ensure recovery from mental health issues and substance use challenges becomes a reality.



For more information or to set up a meet and greet, please call the OIFA administrator at 1-602-255-8605

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 network

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs maintain and monitor a network of participating care providers including physicians, hospitals, skilled nursing facilities (SNFs), ancillary providers and other health care providers through which members obtain covered services.

Members using this UHC Dual Complete AZ-Y001 must choose a PCP to coordinate their care. PCPs are the basis of the managed care philosophy.

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs work with contracted PCPs who manage the health care needs of members and arrange for medically necessary covered medical services. You may, at any time, advocate on behalf of the member without restriction to help ensure the best care possible for the member. In particular, you are not prohibited or otherwise restricted from advising or advocating, on behalf of a member who is your patient, for:

- a. The member's health status, medical care or treatment options, including any alternative treatment that may be self-administered.
- b. Any information the member needs to decide among all relevant treatment options.
- c. The risks, benefits, and consequences of treatment or non-treatment; and,
- d. The member's right to participate in decisions regarding their behavioral health care, including the right to refuse treatment, and to express preferences about future treatment decisions.

To help ensure continuity of care, members must coordinate with their PCP before seeking care from a specialist, except in the case of specified services (such as women's routine preventive health services, routine dental, routine vision, and behavioral health). Contracted health care professionals are required to coordinate member care within the care provider network. If possible, all members should be directed to UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs contracted care providers.

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs require prior authorization. The services must be a covered benefit, and the member must be eligible on the date of service.

All out-of-network services will be denied unless prior authorization has been obtained, or services are emergent in nature.

The referral and prior authorization procedures explained in this manual are particularly important to the UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs. Understanding and adhering to these procedures are essential for successful participation. A prior authorization list is available online at UHCprovider.com/azcommunityplan in the Prior Authorization and Notification section.

Occasionally the Dual Complete AZ-S001 and Dual Complete One AZ-Y001 programs distribute communication documents on administrative issues and general information to you and your office staff. It is very important that you and/or your office staff read the newsletters and other special mailings and retain them with this care provider manual, so you can incorporate the changes into your practice.

Participating care providers

Primary care providers

UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs contract with certain care providers whom members may choose to coordinate their health care needs. These care providers are known as PCPs. PCPs are generally physicians of internal medicine, pediatrics, family practice or general practice. However, they may also be other provider types who accept and assume PCP roles and responsibilities. All members must select a PCP when they enroll in UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 programs and may change their designated PCP once a month.

Specialists

A specialist is any licensed participating care provider (as defined by Medicare) who provides specialty medical services to members.

A PCP may refer a member to a specialist as medically necessary.

Care provider privileges

To help our members get access to appropriate care, you must have privileges at applicable participating facilities or arrangements with a participating practitioner to admit and provide facility services. This includes but is not limited to full admitting hospital privileges, ambulatory surgery center privileges, and/or dialysis center privileges.

Autodialer, artificial or pre-recorded voice technology

You consent to UnitedHealthcare and its affiliates calling the phone number(s) you provided using an auto-dialer and/or artificial or prerecorded voice technology. To opt out of these outreaches, submit the **Telephone Consumer Act (TCPA) Opt Out form**. The Provider TCPA Opt Out will be processed within 10 business days and registered until such time that the phone number is opted back in for such communications. Should you decide to opt back into receiving such outreaches after opting out, submit the **TCPA Provider Opt In form** and it will be processed within 10 business days and your phone number will be removed from the do not call registry.

For additional TCPA Opt Out assistance, please contact Provider Services. Connect with us 24/7 from the **UnitedHealthcare Provider Portal**. For chat options and contact information, visit UHCprovider.com/contactus.

Community Resource Guide

UnitedHealthcare Community Plan is committed to helping people live healthier lives. To support this commitment, the Community Resource Guide is available.

This guide lists community organizations across Arizona and highlights local resources that may help address health-related social needs.

You can access the Community Resource Guide here: **AZ Community Resource Guide AHCCCS**.

Chapter 2: Covered services

Summary

Medicare Cost-sharing for Members Enrolled in UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001 (Costs will vary significantly based on the member's category of Medicaid assistance). Dual programs include QMB Plus, FBDE and SLMB Plus.

For Ancillary benefit planning	Medicaid eligibility status	Does Medicaid cover Part A premium?	Does Medicaid cover Part B premium?	Does Medicaid cover Part D premium?	Does Medicaid cover Medicare deductibles, copays, co-insurance?	Does the member have full Medicaid benefits?
Full Dual	QMB Plus (02)	Qualified Medicare Beneficiary w/full Medicaid.	Yes (when applicable.) Yes	No*	Yes	Yes
Full Dual	FBDE (08)	Full benefit Dual eligible.	Yes (when applicable.) Varies by state.	No	Yes	Yes
Partial Dual	SLMB Plus (04)	Specified low income Medicare beneficiary w/full Medicaid.	Yes (when applicable.) Yes	No*	Yes	Yes
Partial Dual	QMB Only (01)	Qualified Medicare beneficiary.	Yes (when applicable.) Yes	No*	Yes	No
Partial Dual	SLMB Only (03)	Specified low income Medicare beneficiary.	No Yes	No*	No	No
Partial Dual I	QDWI (05)	Qualified disabled working individual.	Yes (when applicable.) No	No	No	No
Partial Dual	QI (06)	Qualified individual.	No Yes	No*	No	No

*QMBs, SLMBs and QIs are automatically enrolled in the low income subsidy program to cover Part D premium costs and will not have Part D premiums.

Medicaid (Medicaid contractor) pays the Medicare cost-sharing (coinsurance, deductible, or copayments except Part D), up to the lesser of the Medicare or Medicaid rate, for Medicare-covered benefits except prescription drug copayments (unless institutionalized and then no prescription drug copayments).

Supplemental benefits (dental, vision, product catalog, etc.) are covered by the Medicare Plan. There is no Medicare cost-sharing. Once a supplemental benefit is exhausted, if it's not covered by Medicare, the member is responsible for payment unless otherwise covered by Medicaid.

NON-QUALIFIED MEDICARE BENEFICIARY DUALS (ACOM 201)

A Non-QMB dual who receives covered services under A.A.C. R9-22 Article 2 or A.A.C. R9-28.

Article 2 from an in-network care provider is not liable for:

1. Any applicable Medicare Cost Sharing (deductible, coinsurance, or copayment) amounts associated with those services, or

2. For any balance of billed charges, unless services have reached the limitations specified within A.A.C. R9-22 Article 2.
3. a.) When a Non-QMB Dual elects to receive services that are covered by both Medicare and Medicaid, from an out-of-network care provider, the individual is responsible for any Medicare deductible, coinsurance, or copayment amounts unless the service is emergent, or, for non-emergency services, the care provider has obtained a signed document from the member to pay for the services as required in A.A.C. R9-22-702.
4. Contractor Payment Responsibilities (In Network)
5. a.) In accordance with A.A.C. R9-29-303, when a Non-QMB Dual receives covered services from an in-network care provider, and the covered service is provided up to the limitations as specified in A.A.C. R9-22 Article 2, the individual is not liable or any balance of billed charges.

Non-qualified Medicare beneficiary Duals (In-Network) Medicare cost sharing requirements	
When the service is covered by: The contractor shall not pay:	When the service is covered by: The contractor shall pay:
Medicare only:	(Subject to the limits specified in this policy)
Any Medicare deductible, coinsurance, or copayment amounts	Medicaid only: The care provider in accordance with the contractor's subcontract by both Medicare and Medicaid. The lesser of the following (unless the contractor's subcontract with the care provider sets forth different terms): a.) The Medicare deductible, coinsurance and/or co-payment amounts, or b.) Any remaining Medicare cost sharing amount after the Medicare payment amount is deducted from the care provider's subcontracted rate (The contractor's contracted rate).

Non-qualified Medicare beneficiary Duals (Out-of-Network) Medicare cost sharing requirements	
When the service is covered by:	The contractor shall pay: (Subject to the limits specified in this policy)
	a.) In accordance with A.A.C. R9-29-303, when a Non-QMB Dual receives covered services from an out of network care provider, the following applies.
Medicare only.	Has no responsibility for payment.
Medicaid only AND the contractor has not referred the member to the care provider OR has not authorized the care provider to render services and the services are not emergent.	Has no responsibility for payment
Medicaid only AND the contractor has referred the member to the care provider, or has authorized the care provider to render services, or the services are emergent	Shall pay in accordance with the requirements of A.A.C. R9-22-705.
By both Medicare and Medicaid AND the contractor has not referred the member to the care provider, OR has not authorized the care provider to render services and the services are not emergent	Has no responsibility for payment

2. Member

If a member has been advised of the contractor's (Medicaid HMO) network, and the member's responsibility is delineated in the member handbook, and the member elects to go out-of-network, the member is responsible for paying the Medicare cost-sharing amount. (Emergent care, pharmacy, and other prescribed services are the exceptions.) This member responsibility must be explained in the contractor's (Medicaid HMO) Member handbook.

Medicaid benefits

Information for members with Medicare and Medicaid

UHC Dual Complete AZ-Y001 is a Full Dual-Eligible Special Needs Plan (D-SNP). It is designed for persons entitled to both Medicare and Medicaid. If members have both Medicare and Medicaid, services are paid first by Medicare and then by Medicaid. Medicaid coverage depends on income, resources and other factors.

Following are the categories of people who may enroll in UHC Dual Complete AZ-Y001:

Qualified Medicare Beneficiary Plus

QMB+ You get Medicaid coverage of Medicare cost-share and are eligible for full Medicaid benefits. Medicaid pays your Part A and Part B premiums, deductibles, coinsurance and copayment amounts.

Specified Low-Income Medicare Beneficiary Plus

SLMB+ Medicaid pays your Part B premium and provides full Medicaid benefits.

Full Benefits Dual Eligible

FBDE Medicaid may provide limited assistance with Medicare cost-sharing. Medicaid also provides full Medicaid benefits.

If Specified Low-Income Medicare Beneficiary Plus or Full Benefits Dual Eligible:

SLMB+ or FBDE Members may be eligible for full Medicaid benefits. At times, they may also be eligible for limited assistance from the Arizona Health Care Cost Containment System (AHCCCS) in paying Medicare cost share amounts. Generally, cost-share is 0% when the service is covered by both Medicare and Medicaid.

There may be cases where members pay cost-sharing when a service or benefit is not covered by Medicaid.

If category of Medicaid eligibility changes, cost-share may also increase or decrease. Members must rectify Medicaid enrollment to continue to receive Medicare coverage.

UHC Dual Complete AZ-Y001

Eligible categories

Qualified Medicare Beneficiary Plus

QMB+ Medicaid pays the Medicare cost-share. This includes deductibles, coinsurance and copayment amounts. Medicaid also pays for QMB+ Part A and Part B premiums. QMB+ members have full Medicaid benefits. QMB+ beneficiaries have \$0 cost-share except for Part D prescription drug copays.

Full Benefit Dual Eligible members

FBDE members have full Medicaid benefits. Medicaid pays the cost-share for covered services rendered by a participating Medicare care provider. At times, members may be eligible for limited assistance from the AHCCCS in paying Medicare cost-share amounts. Generally, members' cost-share is 0% when both Medicare and Medicaid cover the service.

There may be cases where members have to pay a cost-share when Medicaid and Medicare does not cover a service or benefit.

Specified Low-Income Medicare Beneficiary

SLMB+ Medicaid pays members' Part B premium and provides full Medicaid benefits.

What that means to our members:

If they are a QMB+ beneficiary:

They have 0% cost-share, except for Part D prescription drug copays.

If members are a Specified Low-Income Medicare Beneficiary Plus or Full Benefits Dual Eligible Beneficiary:

SLMB+ or FBDE They are eligible for full Medicaid benefits. At times, they may also be eligible for limited assistance from the AHCCCS in paying Medicare cost-share amounts. Generally, the cost share is 0% when both Medicare and Medicaid cover the service. There may be cases where members have to pay a cost-share when Medicaid does not cover a service or benefit.

How to read the Medicaid benefit chart

The following benefits are covered by Medicaid. For each of the following benefits listed, you can see what Arizona Health Care Cost Containment System (AHCCCS) covers and what our plan covers. If a benefit is used or not covered by Medicare, then Medicaid may provide coverage. This depends on the member's type of Medicaid coverage. Additional information regarding benefits can be found online at UHCprovider.com > Resources > Choose a Location > Arizona > Medicare > **Arizona Dual Complete® Special Needs Plans**.

Arizona Health Care Cost Containment System Medicare Advantage Special Needs Plans for Dual eligible members 2026 benefits

In order for you to better understand your health care options, the following chart notes your charges for certain services under the Arizona Health Care Cost Containment System (Medicaid) as an individual who has both Medicare and Medicaid. Your Medicare cost sharing responsibility is based on your level of Medicaid eligibility.

- Qualified Medicare Beneficiary (QMB) – \$0. Your Medicare cost sharing amounts will be paid by your Medicaid Health Plan unless otherwise noted below
- Non-QMB with Medicare Parts A and B – Your Medicare cost sharing amounts will be paid by your Medicaid Health Plan only when the benefit is also covered by Medicaid

Accute and Long- term care Medicaid programs (1)	As an AHCCCS – QMB Dual Eligible – you pay:	As an AHCCCS – Non-QMB Dual Eligible – you pay:
Inpatient hospital stay	\$0	\$0
Inpatient behavioral health care stay	\$0	\$0
Nursing facility services	\$0	\$0
Home health care visit	\$0	\$0
Primary care physician (PCP) visits	\$0	\$0 for well visits, and \$0 to \$4 for other visits depending on eligibility (2) for ages 21 and over (2). \$0 for ages 20 and under.
Specialist physician visit	\$0	\$0 for well visits, and \$0 to \$4 for other visits depending on eligibility (2) for ages 21 and over. \$0 for ages 20 and under.
Medicare-covered services, including chronic/complex case management, etc.	\$0	\$0 for ages 20 and under. Not covered for ages 21 and over.
Chiropractic visits	\$0	\$0 for ages 20 and under; \$0 to \$2.30, for ages 21 over depending on eligibility (2), for up to 20 medically necessary visits beginning October 1st of each year, (additional visits may be authorized if medically necessary).

Accute and Long-term care Medicaid programs (1)	As an AHCCCS – QMB Dual Eligible - you pay:	As an AHCCCS – Non-QMB Dual Eligible - you pay:
Podiatry services visit	\$0	\$0
Outpatient behavioral health care visit	\$0	\$0
Outpatient substance abuse care visit	\$0	\$0
Ambulatory surgical center or outpatient hospital facility visit	\$0	\$0 to \$3 depending on eligibility (2) for ages 21 and over. \$0 for ages 20 and under.
Ambulance services	\$0	\$0
Emergency services	\$0	\$0
Urgently needed care visit	\$0	\$0 to \$4 depending on eligibility (2) for ages 21 and over. \$0 for ages 20 and under.
Outpatient occupational/physical/speech therapy visit	\$0	\$0 to \$3 depending on eligibility (2) for ages 21 and over. \$0 for ages 20 and under.
Durable medical equipment	\$0	\$0
Prosthetic devices	\$0	\$0. Lower limb microprocessor controlled limb or joint not covered for ages 21 and over.
Diabetes self-monitoring training & supplies (when provided as part of a PCP visit)	\$0	\$0
Community health worker (CHW) visit	\$0	\$0
Diagnostic tests, x-rays, and laboratory s(including COVID-19 diagnostic & testing services)	\$0	\$0
Colorectal screening	\$0	\$0

Accute and Long-term care Medicaid programs (1)	As an AHCCCS – QMB Dual Eligible - you pay:	As an AHCCCS – Non-QMB Dual Eligible - you pay:
Flu and pneumonia vaccines	\$0	\$0
Screening mammogram	\$0	\$0
Pap smear and pelvic exam	\$0	\$0
Prostate cancer screening	\$0	\$0
Renal dialysis or nutritional therapy for end-stage renal disease	\$0	\$0
Prescription medications (3)	\$0	\$0 to \$2.30 depending on eligibility (2) for ages 21 and over. \$0 for ages 20 and under.
Hearing exams, routine hearing tests, and fitting evaluations for a hearing aid	\$0 for ages 20 and under. Not covered for ages 21 and over.	\$0 for ages 20 and under. Not covered for ages 21 and over.
Hearing aids	\$0 for ages 20 and under. Not covered for ages 21 and over.	\$0 for ages 20 and under. Not covered for ages 21 and over.
Cochlear implants	\$0	\$0
Routine eye exam, eyeglasses, contact lenses, lenses and frames	\$0 for ages 20 and under. Not covered for ages 21 and over unless following cataract surgery.	\$0 for ages 20 and under. Not covered for ages 21 and over.
Adult emergency dental services	\$0 for ages 21 and over. Services subject to a \$1,000 limit per each 12 month period beginning October 1st of each year.	\$0 for ages 21 and over. Services subject to a \$1,000 limit per each 12 month period beginning October 1st of each year.
Non-emergency medically necessary transportation	\$0	\$0

Long term care Medicaid programs only (1)	As an AHCCCS – QMB Dual Eligible - you pay:	As an AHCCCS – Non-QMB Dual Eligible - you pay:
Nursing facility services	Cost sharing determined by AHCCCS.	Cost sharing determined by AHCCCS.
Respite services	\$0. Subject to a 600 hour limit per each 12 month period beginning October 1st of each year.	\$0. Subject to a 600 hour limit per each 12 month period beginning October 1st of each year.
Home and Community-Based Services	Member contribution determined by AHCCCS.	Member contribution determined by AHCCCS.
Adult preventive dental services (4)	\$0 for ages 21 and over. Services subject to a \$1,000 limit per each 12 month period beginning October 1st of each year.	\$0 for ages 21 and over. Services subject to a \$1,000 limit per each 12 month period beginning October 1st of each year.

1. Acute Medicaid Programs include AHCCCS Complete Care (ACC), ACC with a Regional Behavioral Health Agreement (ACC-RBHA), and the Mercy Care Department of Child Safety Comprehensive Health Plan (Mercy Care DCS CHP). Long Term Care Medicaid Programs include Elderly and/or Physically Disabled (E-PD) and the Division of Developmental Disabilities (DDD).
2. See the AHCCCS website for additional beneficiary cost sharing, co-payment and benefits related information.
3. Medicare Part D co-payment amounts are the sole responsibility of the beneficiary. AHCCCS health plans cannot assist with the payment of these amounts, except for behavioral health medications for those beneficiaries determined to be Seriously Mentally Ill (SMI) utilizing allowable Non-Title XIX/XXI funding.
4. In addition to Adult emergency dental services described above.

Arizona Health Care Cost Containment System
Additional services available through UHC Dual Complete AZ-S001 and Dual Complete AZ-Y001

Benefit	QMB or QMB + members pay	SLM B+ or FBDE members pay:	UHC Dual Complete AZ-S001 and UHC Dual Complete AZ-Y001
Additional dental services	No coverage.	No coverage.	Covered.
Additional foot care	No coverage.	No coverage.	Covered.
Additional hearing services	No coverage.	No coverage.	Covered.
Healthy food	No coverage.	No coverage.	Covered.
Over-the-counter items	No coverage.	No coverage.	Covered.
Utilities and in home supports	No coverage.	No coverage.	Covered.
Transportation (routine)	No coverage.	No coverage.	UHC Dual Complete AZ-S001: No coverage. UHC Dual Complete AZ-Y001: Covered.
Additional vision services	No coverage.	No coverage.	Covered.

Members who are enrolled in UHC Dual Complete programs may also be covered by UnitedHealthcare Community Plan AHCCCS (Medicaid) benefits. Members should refer to their Medicaid Member Handbook for further details on Medicaid benefits. Members who are enrolled in another AHCCCS (Medicaid) plan must coordinate their benefits with that plan.

Prior authorization

Services requiring prior authorization are available on [UHCprovider.com](https://uhcprovider.com) in the Prior Authorization and Notification section (periodically updated). The presence or absence of a procedure or service on the list does not define whether coverage or benefits exist for that procedure or service. A facility or practitioner must contact UHC Dual Complete programs for prior authorization. Submit requests for prior authorizations using the Prior Authorization and Notification tool on the UnitedHealthcare Provider Portal or by calling to the Prior Authorization Department at **1-800-445-1638**.

Referral guidelines

PCPs are generally responsible for initiating and coordinating coverage for medically necessary services beyond the scope of their practice for Dual Complete programs members if a **contracted care provider** is not available. A referral to a non-contracted care provider may be requested, but UnitedHealthcare Community Plan must authorize the referral. PCPs monitor the progress of referred members' care and see that members are returned to the PCP's care as soon as possible.

All referrals to non-contracted care providers require the completion of a referral form.

If a contracted care provider is not available to provide services a referral can be completed. Referrals are to be written on the same UHC Dual Complete programs referral form you use for UnitedHealthcare Community Plan Medicaid members. Prior authorization is required when services are performed by a non-contracted care provider.

The PCP is to complete, date, and sign (a signature stamp is acceptable) the referral form.

Forward a copy of the referral form to the non-contracted specialist. Referrals are limited to an initial consultation and up to 2 follow-up visits. Follow-up visits must be completed within 180 calendar days from the date the referral is signed and dated.

Referrals for hematology/oncology, radiation oncology, gynecology oncology, allergy, orthopedic services, and nephrology are valid for unlimited visits within the 180 day timeframe

Emergency and urgent care

Definitions

An emergency medical condition is a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) such that a prudent layperson, with an average knowledge of health and medicine could reasonably expect the absence of immediate medical attention to result in:

- Serious jeopardy to the health of the individual or, in the case of a pregnant woman, the health of the woman or her unborn child
- Serious impairment to bodily functions
- Serious dysfunction of any bodily organ or part

Emergency services are covered inpatient and outpatient services that are:

- Furnished by a care provider qualified to furnish emergency services
- Needed to evaluate or stabilize an emergency medical condition

Members with an emergency medical condition should be instructed to go to the nearest emergency care provider.

Members who need urgent (but not emergency) care are advised to call their PCP, if possible, prior to obtaining urgently needed services. However, prior authorization is not required.